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European Patent Application No. 17821262.7

TESARO INC.

Our Ref: N421104EP RCS/JMS

I write in response to the communication under Article 94(3) EPC dated 6 May 2021. Please replace claims 1 to 9 currently on file with the attached amended claims 1 to 8. Please also replace the description (pages 1-79) currently on file with the attached amended description pages 1-78.

To assist the examiner, I also attach a copy of the amended pages showing the changes made in marked-up form.

Amendments

Claim 1 has been amended to specify that the therapeutic agent is niraparib. Basis for this amendment may be found in previous claim 4, and at line 8 of paragraph [0008] on page 3 of the application as filed.

Previous claim 4 has been deleted, and previous claims 5 to 9 have been renumbered accordingly.

Previous claim 9 (present claim 8) has been corrected to specify that the cancers are platinum sensitive. Basis for this amendment may be found in claims 23 to 28 of the application as originally filed.

The description has also been amended for conformity with the claims, and to remove statements which purport to incorporate by reference the entire content of cross-referenced documents.

Novelty (Art 54 EPC)

The outstanding objection of lack of novelty is based on D1.

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Claim 1 has now been amended to specify that the anti-PARP therapeutic agent is niraparib. D1 does not disclose niraparib. The claimed subject matter is therefore novel.

Inventive Step (Art 56 EPC)

Technical problem and solution

The examiner has identified D1 as the closest prior art. D1 describes the results of a Phase 2 clinical study with olaparib in patients having platinum-sensitive, relapsed, high-grade serious ovarian cancer. As explained above, the present application differs from D1 in at least the use of niraparib, rather than olaparib.

The technical effect of this difference is that use of the specific drug niraparib enables efficacy in the treatment of patients having an absence of BRCA1 and/or BRCA2 mutation. This is a recalcitrant patient group which had not been shown to be effectively treated with the drug disclosed in D1.

In this regard, those of skill in the art would be well aware at the priority date of the present application that olaparib was not approved for the treatment of cancer patients having an absence of mutation in BRCA1 and/or BRCA2, as required by the present claims. In this regard, we attach the European Public Assessment Report (EPAR) published by the European Medicines Agency for Lynparza (olaparib). This document issued in 2014, before the priority date of the present application in 2016.

Section 4.1 of the EPAR for Lynparza (olaparib) clearly states that Lynparza is for use only in patients with BRCA mutations:

Lynparza is indicated as monotherapy for the maintenance treatment of adult patients with platinum-sensitive relapsed BRCA-mutated (germline and/or somatic) high grade serous epithelial ovarian, fallopian tube, or primary peritoneal cancer who are in response (complete response or partial response) to platinum-based chemotherapy. (emphasis added)

Therefore, at the priority date of the present application, it was clearly established in the art that olaparib was not to be used in the treatment of cancer in patients having an absence of BRCA1 and/or BRCA2 mutation. The deficient activity of olaparib in such patients is, in fact, discussed in the present application. Thus, paragraph [00150] of the application explains that:

*“the olaparib clinical data demonstrated that PARP inhibitors **would not be beneficial** to prolong progression free survival in the treatment of cancer characterized by the **absence of mutations in BRCA1 or BRCA2.**”* (emphasis added).

In contrast, the examples of the present application demonstrate that niraparib is effective in treating cancer in patients who have an absence of a mutation in BRCA1 and/or BRCA2. In particular, it can be seen from the application that progression-free survival (PFS) was significantly longer for patients who received niraparib than for patients who received placebo across all patient groups (paragraph [00223]).

Paragraph [00225] goes on to state that niraparib showed statistical significance in the overall non-germline BRCA mutant cohort where the median PFS for patients treated with niraparib was 9.3 months compared with 3.9 months for control.

Further, we attach the European Public Assessment Report (EPAR) published by the European Medicines Agency for Zejula (niraparib). In contrast, to the EPAR for olaparib, the corresponding section 4.1 of the EPAR for Zejula (niraparib) does not contain any limitation on the BRCA mutation status of the patients to be treated:

Zejula is indicated:

- *as monotherapy for the maintenance treatment of adult patients with advanced epithelial (FIGO Stages III and IV) high-grade ovarian, fallopian tube or primary peritoneal cancer who are in response (complete or partial) following completion of first-line platinum-based chemotherapy.*
- *as monotherapy for the maintenance treatment of adult patients with platinum-sensitive relapsed high grade serous epithelial ovarian, fallopian tube, or primary peritoneal cancer who are in response (complete or partial) to platinum-based chemotherapy.*

The effective treatment of patients without mutations in BRCA1 or BRCA2 by niraparib, as compared with olaparib is referenced in the application as filed. Paragraph [00224] of the description explains that that the “*results [with niraparib] were markedly better than those from “Study 19” (N Engl J Med. 2012; 366(15):1382-1392)*”. “Study 19” is document D1 (Ledermann et al) on file in these proceedings.

Therefore, the technical problem to be solved by the present application is the provision of a PARP inhibitor which provides an effective treatment in patients having an absence of a mutation in BRCA1 and/or BRCA2. The solution is niraparib for use as set out in the claims of the present application.

Obviousness

There is nothing in D1 which remotely suggests that an effective treatment against the specific patient population specified in claim 1 of the present application could be achieved by use of niraparib in place of olaparib. The claimed subject matter does therefore involve an inventive step.

Clarity

Previous claim 4 has been deleted, so the objection with respect to this claim no longer applies.

Conclusion

It is believed that all the points raised in the examination report have been met and that this application is now in allowable form. Accordingly, I look forward to receiving a communication under Rule 71(3) EPC in due course.

In the event that there are any substantive matters outstanding, the examiner is requested to issue a further written communication or, if more appropriate, to contact me by telephone or at mail@jakemp.com.

In the unlikely event that the examiner should consider rejection of the application I request, purely as a precaution, that oral proceedings be appointed.

Electronically Signed
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